

**Beth Israel Deaconess Medical Center
BIDMC Manual**

Title: Organ Donation after Circulatory Death (DCD)

Policy #: PR-13

Purpose: To provide a clinically and ethically sound procedure for the procurement of organs for transplantation after circulatory of death.

Policy Statement:

Definition: “Donation after Circulatory Death” (DCD) is a donor whose death is defined by “irreversible cessation of circulatory and respiratory function”.

Goals:

- I. To honor the wishes of the patient in all end-of-life matters.
- II. To ensure the comfort and dignity of the patient (donor).
- III. To optimize the patient’s family’s experience throughout the organ donation process.
- IV. To maintain separation between critical care services and transplant services during the process of organ donation after circulatory of death.
- V. To ensure that organ viability is optimized.

Procedure(s) for Implementation:

This policy references the following policies:

1. [**CG-28 Guidelines for Interdisciplinary Comfort-Focused End-of-Life Care**](#)
2. [**CCG-#2 Ventilator Discontinuation for Transition to Comfort at End of Life**](#)
3. [**CC-#15 Train of Four Peripheral Nerve Stimulator**](#)

I. Care of Potential Donor:

- A. The patient will continue to be under the care of the attending physician of record and/or the critical care physician.
- B. The critical care team will work with the patient, family/next-of-kin or other patient surrogate decision maker in order to best respect, honor, and implement the patient’s end-of-life care.
- C. The critical care team will counsel the family on end-of-life care and coordinate the end of life and New England Donor Services (NEDS) referral process in a manner that minimizes the risk for patient, family, or critical

care team distress.

- D. Administration of comfort care order sets, including opiates, sedation, and/or other analgesics, will occur at the discretion of the critical care team in order to maintain patient comfort and dignity at the time of death. Palliative care consultation could be (but is not required to be) considered for symptom management in all end-of-life cases.

II. Referral/Criteria:

- A. The patient must be on a ventilator. (**NOTE:** ECMO is not excluded.)
- B. End-of-life discussion with the patient and/or patient's surrogate decision maker(s) is **planned**.

An initial referral to the New England Donor Services (NEDS) will be made by hospital staff, prior to any end-of-life discussions when possible, including discussions of de-escalation of care. The clinical NEDS coordinator will review case with a member of the patient's care team to determine suitability for organ donation and likelihood of death within prescribed time frame.

III. Consent Process:

- A. Following the patient's or appropriate surrogate decision maker's documented decision to withdraw life sustaining medical treatment:
 - 1. In the absence of a legal Donor Designation (*Donor card signed by the decedent, registration with a legally recognized donor registry, or other documentation such as a will or health care proxy form*) NEDS Coordinator obtains written consent for organ and tissue recovery from the patient's appropriate legal agent or next-of-kin. NEDS will provide information about the donation process and support the family as needed.
 - 2. If a Donor Designation has been identified, the patient or appropriate surrogate decision maker must agree to the manner of withdrawal of life-sustaining treatment allowing for organ recovery. (See Attachment D, "Consent for Donor Management".) The physician or APP will complete Attachment D, "Consent for Donor Management". NEDS will provide information about the donation process and support the family as needed in their preferred language with the use of medical interpreters when indicated.
 - 3. Additional support staff including social work, pastoral care, interpreter services and palliative care services will be contacted as indicated.
 - 4. Consent for DCD Conditions involving a potential DCD donor being medically treated/supported in a conscious mental state will require that the organ procurement organization (OPO) confirms that the healthcare team has assessed the patient's capacity to make withdrawal/support and other medical decisions.

- B. Patient or appropriate surrogate decision maker must sign a separate hospital consent form for any pre-mortem procedures and/or medications. See Attachment D, "Consent for Donor Management".

IV. Post Consent Planning:

- A. The ICU Physician will pre-screen all consented cases for clearance with the Medical Examiner, regardless of circumstances of death. If appropriate, call the Boston Medical Examiner Office 24-hour number: 800-962-7877.
- B. Before withdrawing life-sustaining treatment, a team huddle will be held with NEDS and BIDMC staff several hours prior to extubation to (see Attachment B: DCD Workflow Checklist):
1. Determine roles and responsibilities of patient care team. A Critical Care fellow or APP may be present for withdrawal of mechanical ventilation and declaration of death in the DCD setting under the supervision and discretion of the critical care attending with the expectation that there will be direct attending oversight. **Attending must be present for extubation; a clinician designee must be present from extubation to the 2-hour time limit.**
 2. Verify patient identification, surrogate designation, donation consent and confirm code status in electronic orders as DNR/DNI and CMO. **Unlike Donation after Brain Death (DBD), patients consented for DCD are not required to be full code and may be DNR/DNI at the patient family or surrogate discretion.**
 3. Determine the location for withdrawal of life-sustaining treatment. A location most likely to reduce potential for patient and family distress and maximize successful organ procurement should be selected. **Organ viability and transplantation would be optimized with withdrawal of life-sustaining treatment in the operating room (OR), which will be the standard location.** However, shared decision-making with the patient's surrogate decision maker or next-of-kin should be made to support the optimal location.
 4. Determine OR time utilization and which OR will be utilized for procurement. When a known case is anticipated, the NEDS coordinator will call the charge nurse and provide the anticipated date/time.
 5. Determine the location for final goodbyes between the patient and family / next of kin. The standard location for final goodbyes will be the intensive care unit (ICU) or post anesthesia care unit (PACU), but shared decision-making with the patient's surrogate decision maker or next-of-kin should be made to support optimal location. Should families wish to be present for withdrawal of care in the OR, this will be coordinated with the NEDS staff and social work and limited to 2 family members (exceptions for > 2 family members will be discussed with

NEDS Staff and responsible attending physician).

6. Discuss roles and responsibilities of NEDS staff and organ recovery teams. All team members/roles will be clear and ensure everyone understands that the patient does not meet brain death criteria.
 7. Notify Public Safety about DCD donation timing/plan.
 8. Review the process for withdrawing life-sustaining treatment or ventilator support. (Refer to CCG–#2 Ventilator Discontinuation for Transition to Comfort at End of Life).
 9. The hospital's plan for continued patient care if the patient does not become a donor, and appropriate communication with the surrogate decision maker or next-of-kin is ensured (Refer to OPTN policy 2.15.f #4).
 10. If needed, consult Palliative Care for any questions about care (pager #32502). Refer to CG-28 for guidelines.
- C. BIDMC will utilize the accepted method of terminating life-sustaining treatment as determined by the attending physician and/or BIDMC policy. The attending physician, ICU attending, or his/her designee or Advanced Practice Provider (APP) will pronounce death. No recovery personnel (surgeons or other recovery practitioners) may be present for the withdrawal of life-sustaining treatment or ventilator support (Refer to OPTN policy 2.15.Fs). The physician who declares the patient dead must not be directly associated with the BIDMC transplant team or simultaneously involved in the care of a potential transplant recipient. No member of the organ recovery team or OPO staff may participate in the guidance or administration of end-of-life care, or the declaration of death.
1. Ensure no neuromuscular blockade (NMB), or discontinue any neuromuscular blockade, at least 1 hour prior to extubation. Assess for return of neuromuscular function prior to extubation (see CC-#15) or consider NMB reversal prior to palliative extubation (see CCG-#2).
- D. Discontinue propofol (except when using for refractory seizures). For patients on existing infusions of opioids and benzodiazepines, consider rationale for ongoing need. If not compromising patient comfort, decrease or discontinue prior to withdrawal of ventilator support. If needed for patient comfort, it is acceptable to continue continuous infusions of opiate and/or sedation during CMO for DCD.
- E. Ensure licensed practitioner writes medication orders to manage distressing symptoms including pain, agitation, air hunger, and noisy secretions, and that medication is on-hand prior to extubation (e.g. opioid plus benzodiazepine; consider anticholinergic agents).
- F. Patients should receive comfort-focused medications at the time of extubation per our comfort-focused care guidelines (refer to CG-28).
- G. Removal of supplemental oxygen may be considered. Refer to CG-28.

V. Donor Management (Under direction of NEDS, laboratory work will be started after consent for organ donation is obtained and with a physician's order)

- A. Evaluation: Stat Lab and diagnostic tests will be done including some or all of the following: electrolytes, glucose, PT/PTT, ABO, ABG, CBC with differential, lactate, BUN, creatinine, LFT's, total and direct bilirubin, albumin, amylase, lipase, HgbA1c, urinalysis, blood, urine, and sputum culture and sputum gram stain, chest X-ray, bronchoscopy
- B. Blood for Stat serology and tissue typing tests will be done. NEDS coordinator is responsible for arranging transportation and testing.
- C. Patient is to be hemodynamically supported including pressor and/or fluids until CMO and withdrawal of support is initiated.
- D. Abnormal parameters to be treated:
 - 1. Electrolyte abnormalities - change IV solutions, rates, and adjust additives.
 - 2. ABG abnormalities - alter ventilator settings
 - 3. Low hematocrit – transfuse per hospital protocol
 - 4. Coagulopathy - warm patient to 37 degrees, give FFP
 - 5. Low urine output - give fluids, mannitol, or furosemide
 - 6. Hypotension - give fluids and / or vasopressors
- E. Pre-mortem procedures and/or medications requiring signed BIDMC consent form, may include but are not limited to:
 - 1. Insertion of arterial line
 - 2. Bronchoscopy for lung evaluation
 - 3. Administration of comfort care order sets, including opiates, sedation, and/or other analgesics, may occur, at the discretion of the attending physician, if the purpose is to benefit the dying patient by easing discomfort at the time of death, even if the order sets may hasten the death.
 - 4. Administration of anticoagulants may be undertaken once care is transitioned to CMO and if agreed upon by NEDS. Specific informed consent must be obtained for its use from the patient or the patient's surrogate decision maker. Typically, heparin will be administered just prior to extubation with the usual dose being 30,000 units (see Attachment B: DCD Workflow Checklist).
 - 5. Administration of vasodilators should be carefully considered only after care is transitioned to CMO and if agreed upon by NEDS, before the patient's heart stops, and before the patient is declared dead and only if the patient, or the patient's surrogate decision maker has given specific informed consent for its administration.

VI. Patient Transport and Extubation:

- A. The patient should be prepared for transport according to the DCD Workflow Checklist (see Attachment B: DCD Workflow Checklist).
 1. Patient prep and draping prior to extubation is the typical workflow but should be addressed and confirmed at the time of huddle. Transplant team sterile draping shall not impede the critical care team's ability to assess patient comfort and vital signs with view of the clavicles. Consider the use of translucent drape (loban) for this purpose.
 2. During prep and draping, care must be taken to not obscure view of the patient's clavicles, intercostal muscles, and face or nares as this will limit ICU clinician's ability to assess for signs of pain or respiratory distress.
 3. Refer to CCG-#2 and CG-28 for withdrawal of mechanical ventilation.

VII. Determining Death

The method of declaring circulatory death must comply in all respects with the legal definition of death by an irreversible cessation of circulatory and respiratory functions **before** the pronouncement of death. (See Attachment E, Maastricht Classification – Definition of Controlled DCD Donors, and The Uniform Determination of Death Act).

Five (5) minutes of pulselessness must be observed before death is pronounced and before proceeding with donation. The accepted medical detection standards include five (5) minutes of:

1. Asystole, or
2. Pulseless Electrical Activity (PEA) with zero pulse pressure as determined by monitoring through an arterial catheter or palpating the carotid artery for a full five (5) minutes to confirm PEA

If Arterial Monitoring is present:

Five (5) minutes of continuous zero pulse pressure will be observed by a running Arterial Monitoring waveform. There must be five (5) minutes of observed apnea simultaneous with five (5) minutes of continuous zero pulse pressure.

If Arterial Monitoring is not present:

Electrocardiographic changes consistent with absent heart function by electronic monitoring (i.e. asystole or PEA). There must also be five (5) minutes of observed apnea simultaneous with five (5) minutes of continuous asystole. The carotid artery should be palpated for a full five (5) minutes to confirm PEA.

Determination of death must then be documented by the provider using the Declaration of Death for Donation after Circulatory Death Checklist (Attachment

C).

If the patient continues to breathe and has a sustained pulse and blood pressure for more than two hours, the donation process will be stopped (this time frame may be shortened based on patient's medical condition). This time limit will be made clear to the patient, patient surrogate decision maker, family or next-of-kin during the consent process. If the time limit expires, they will be notified. The care of the patient will remain in the control of the attending physician or ICU attending.

VIII. Post Mortem Procedures

If lungs are to be recovered, additional procedures may be performed, including but not limited to, reintubation and/or chest tube insertion. The reintubation clinical team should be distinct from the team involved in organ donation and declaration of death. The NEDS Coordinator/Transplant Surgeon will be responsible for any of the post mortem procedures which are covered by the NEDS consent.

IX. Financial Considerations

All costs from the time that the family gives permission for donation until the donation occurs, or the patient is no longer a candidate for donation will be the responsibility of NEDS. Also, NEDS will pay for any lab tests or medications that are ordered by the attending physician to assess organ suitability or prepare the patient for organ recovery.

X. Organ Allocation and Distribution

Organ allocation and distribution will take place according to federal guidelines established through the United Network for Organ Sharing (UNOS) and NEDS.

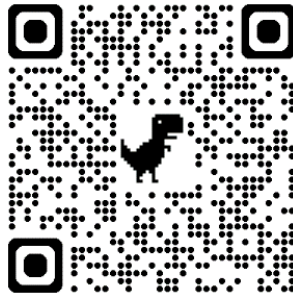
XI. Post Organ Recovery

Following organ recovery, the body will be taken to the morgue. The funeral home or the Office of the Medical Examiner will be notified that the body is available for pick up.

XII. No Organ Recovery

If organ donation will not occur, the family will be notified. NEDS and BIDMC support staff will be notified for ongoing support to the family.

QR Code to Step-by-Step Educational Video:



Link: <https://vimeo.com/1111652418?share=copy#t=373>

Attachment A: Flowchart: Donation after Circulatory Death (DCD)

Attachment B: DCD Workflow Checklist

Attachment C: Checklist: Declaration of Death for Donation after Circulatory Death

Attachment D: Consent for Donor Management

Attachment E: Maastricht Classification - Definition of Controlled DCD Donors and Uniform Determination of Death Act

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☒ **Medical executive Committee: 3/19/25**

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Chair, MEC**

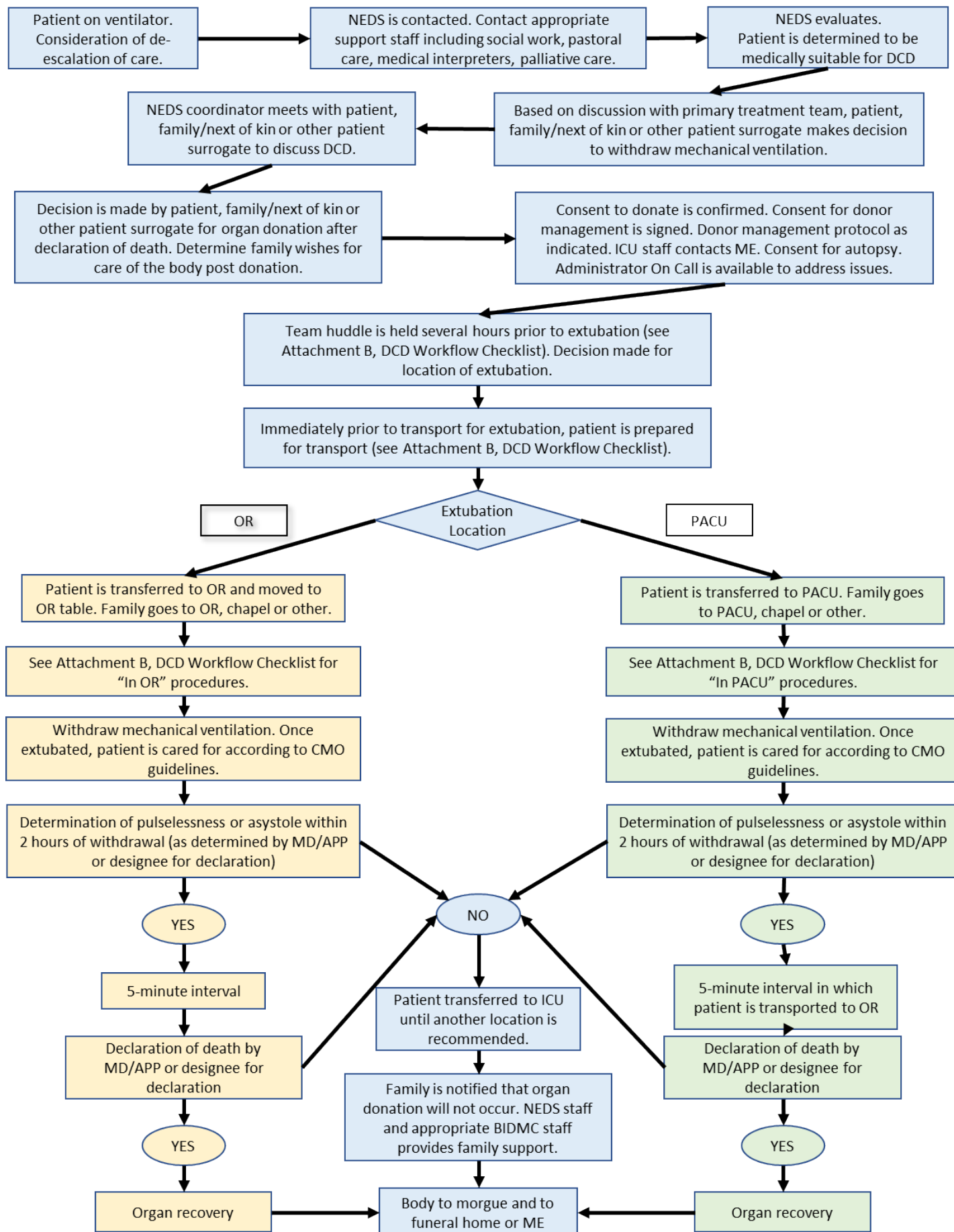
Requestor Name: Todd Sarge, MD

Original Date Approved: 11/06/02

Next Review Date: 3/28

Revised: 3/04, 9/04, 10/04, 10/05, 3/08, 5/11, 12/17, 3/25

Attachment A: Flowchart: Donation after Circulatory Death (DCD)



DCD Workflow Checklist

Team huddle with New England Donor Services (NEDS), ICU staff and OR circulating RN (if possible): several hours prior to withdrawal of care / extubation in the ICU

- ☐ Introduction and role clarification for all staff present at huddle: NEDS Coordinator, ICU nurse, ICU clinicians, OR staff
- ☐ Confirm patient identification, HCP / Guardianship / Next of kin, NEDS donor consent and code status
- ☐ Discuss and confirm plan for location of withdrawal and organ procurement:
 - ☐ Confirm time for withdrawal and location (OR vs PACU) and specific OR number for procurement
 - ☐ Confirm specific organs planned for procurement
 - ☐ In the event of lung procurement, reintubation plan should be established with anesthesia and RT
 - ☐ Confirm whether normothermic regional perfusion (NRP) is being used for procurement
 - ☐ Ensure IV heparin is ordered by provider and delivered to the unit (dose typically in the 30,000-unit range)
 - ☐ Develop plan with NEDS for timing and location of abdominal shave and prep (may be completed by NEDS coordinator or designee)
 - ☐ Confirm whether patient will undergo draping prior to withdrawal / extubation
- ☐ Discuss and confirm family has received explicit communication of the plan and instructions including:
 - ☐ Location for final goodbyes between patient and family and whether family is accompanying patient to OR/PACU
 - ☐ Location where family will wait if not with patient
 - ☐ OR environment (if relevant) including sterility, draping and equipment
 - ☐ Number of family present (typically 2 for OR location) for withdrawal / extubation and instruction and plan for escort with Social Work for family moving in and out of OR / PACU
 - ☐ For OR withdrawals, family will be escorted out of the OR within 2 minutes of pulselessness
 - ☐ Ensure family is aware of plan to return patient to ICU for further comfort-focused care if dying process extends greater than 120 minutes
 - ☐ Offer honor walk to the family and coordinate if within their wishes (to be done by BIDMC staff)
- ☐ Confirm staff accompanying patient to the OR and their roles and responsibilities including:
 - ☐ MD/APP (ICU attending or designee*) for declaration: Clinician name(s)_____
 - ☐ RT for transport and extubation (RT should plan to place patient on transport vent)
 - ☐ ICU RN – to prepare patient for transport including monitoring, meds, etc
 - ☐ NEDS staff (including NEDS family services team member) – will escort families in/out of OR, remain with patient, and notify procurement team(s) of pulselessness and declaration
 - ☐ Procurement team(s) – will prep and drape the patient prior to family's arrival to the OR and should be outside of the OR (clean core or front desk) and not enter the OR until after family has left the OR and patient has been declared by ICU clinician.
 - ☐ Confirm Public Safety is notified and aware of DCD donation timing / plan

***A Critical Care fellow or APP may be present for withdrawal of mechanical ventilation and declaration of death in the DCD setting under the supervision and discretion of the critical care attending with the expectation that there will be direct attending oversight. Attending must be present for extubation; a clinician designee must be present from extubation to the 2-hour time limit.**
- ☐ Determine plan for CMO / withdrawal of care
 - ☐ Discuss patient condition / neurologic status / capacity
 - ☐ Discuss medications to be used
 - ☐ Orders placed for DNR/DNI/CMO/extubation and medications to be used (about 1 hour prior to transport)
 - ☐ CMO order, including extubation, and heparin consent should be printed for NEDS

(Continued on next page)

Attachment B: DCD Workflow Checklist

Preparation for transport: immediately prior to transport for withdrawal of care / extubation

- ☐ RN acknowledges CMO orders
- ☐ All staff accompanying patient to the OR will obtain OR scrubs or coverall provided by OR
 - ☐ If family is accompanying patient, they will also require and be provided appropriate PPE
- ☐ Transfer patient to stretcher (for PACU extubation only)
- ☐ Disconnect any unnecessary infusions or equipment
- ☐ Obtain OR travel monitor from pre-op holding
- ☐ EKG leads are placed posteriorly and connect to OR travel monitor
- ☐ Sheets/blanket to cover patient, or if using gown – ensure it is *not* tied in the back and is draped over patient
- ☐ Draw any additional blood samples as requested by NEDS
- ☐ Bring additional 10mL syringes, flushes, and 30ml luer lock syringe and blunt needle to draw/administer heparin
- ☐ Medications: Narcotics, lorazepam, glycopyrrolate are available in the OR Omnicell. Check with OR staff if any questions regarding which meds are available. Heparin vial must be brought to the OR.

(Continued on next page)

Withdrawal of Care / Extubation Location:

In OR

- ☐ Move patient to OR table
- ☐ RN positions at head of the bed and logs into Epic with standard credentials and signs into home unit
- ☐ RN uses MRN to search for patient under Patient Lookup
- ☐ Consider using anesthesia bolus tubing to administer CMO medications
- ☐ Medications can be found under MAR tab and will be listed as MAR Hold, but can still be documented on
- ☐ Wristband should be exposed on arm for medication scanning
- ☐ Discuss with surgeon and NEDS coordinator patient positioning and draping to ensure patient will be optimized for assessment (for example, head of bed at 30 degrees for respiratory comfort and drapes placed to allow for patient assessment and auscultation). Consider use of translucent drape (Ioban).
- ☐ Ensure clinician for declaration is present for extubation
- ☐ There are 2 Omnicells in the OR (across from room 6 and 16). You can search for patient in the Omnicell and remove medications needed that way. Rover or computer may be used to document medication administration.
- ☐ Consider anticipatory dose of opioid prior to extubation to mitigate risk of post-extubation respiratory distress
- ☐ Heparin IVP administration by RN, timing at discretion of NEDS coordinator and prior to extubation
- ☐ Once extubated, patient is cared for according to CMO guidelines
- ☐ After 5 minutes of asystole or PEA with zero pulse pressure, the patient may then be pronounced in accordance with hospital policy and procurement begins.
- ☐ Once patient is pronounced, RN is not required to stay in room with patient
- ☐ Social work to bring family to quiet space
- ☐ Disconnect IV infusions and document the waste of controlled substances in Omnicell

In PACU

- ☐ Clear path to OR before extubation
- ☐ RN logs into Epic with standard credentials and signs into home unit
- ☐ RN uses MRN to search for patient under Patient Lookup
- ☐ Medications can be found and documented on under MAR tab
- ☐ Wristband should be exposed on arm for medication scanning
- ☐ Discuss with surgeon and NEDS coordinator patient positioning and draping to ensure patient will be optimized for assessment (for example, head of bed at 30 degrees for respiratory comfort and drapes placed to allow for patient assessment and auscultation). Consider use of translucent drape (Ioban).
- ☐ Ensure clinician for declaration is present for extubation
- ☐ Meds may be pulled from PACU Omnicell, and are documented/wasted in PACU. Rover or computer may be used to document medication administration.
- ☐ Consider anticipatory dose of opioid prior to extubation to mitigate risk of post-extubation respiratory distress
- ☐ Heparin IVP administration by RN, timing at discretion of NEDS coordinator and prior to extubation
- ☐ Once extubated, patient is cared for according to CMO guidelines.
- ☐ Once asystolic or in PEA, begin moving to OR and transfer to operating room table
- ☐ Social work to bring family to quiet space
- ☐ OR staff/NEDS to prep and drape
- ☐ After 5 minutes of asystole or PEA with zero pulse pressure, the patient may then be pronounced in accordance with hospital policy and procurement begins.
- ☐ Once patient is pronounced, RN is not required to stay in room with patient
- ☐ Disconnect IV infusions and bring pumps/poles back to unit. Document the waste of controlled substances in the PACU.

Attachment C: Checklist: Declaration of Death for Donation after Circulatory Death

Beth Israel Deaconess Medical Center CHECKLIST: DECLARATION OF DEATH For Donation after Circulatory Death (DCD)	PATIENT IDENTIFICATION
<i>To be completed by medical certifier:</i> Name of Medical Certifier: _____ Name of Attending Physician: _____ Date of Examination: ____ / ____ / ____ Time of Examination: _____ (24-hour clock) Guidelines for DCD / Policy PR-13, in BIDMC Policies and Procedures, reviewed ☐ Consent or Disclosure form for organ donation in chart ☐ DNR order completed ☐ MEO (Medical Examiner's Office) notified (800-962-7877) ☐ Time of Extubation ____ / ____ Date Time (24-hour clock) ☐ Five (5) minutes of continuous asystole/zero pulse pressure: - Start time: _____ / _____ Date Time (24-hour clock) ☐ Asystole - or ☐ Zero Pulse Pressure - Time after 5 minutes: _____ / _____ Date Time (24-hour clock) ☐ Five (5) minutes of asystole / zero pulse pressure confirmed ☐ Patient declared dead at ____ / ____ (time on same running monitor*) Date Time (24-hour clock) *Time of death is the time after the 5-minute hands-off period. III. NOTES/COMMENTS _____ _____ _____ _____ _____ Signature of medical certifier	

Attachment D: Consent for Donor Management

CONSENT FOR DONOR MANAGEMENT

_____ has explained that _____
(Name of Healthcare Provider) (Patient's Name)

has been admitted to the Surgical/Medical/Neuro/Cardiac Intensive Care Service and may be a candidate for organ donation following declaration of death. I understand that the patient's condition requires intensive monitoring and may require aggressive therapy, including invasive procedures designed to optimize the chances for successful organ recovery and donation. I agree these procedures should be performed as considered necessary by the physicians caring for the patient. When possible, the physicians and caregivers responsible for the patient's care will inform me prior to any procedure. I and/or family members can expect regular updates on the patient's condition and the nature of the care being provided to the patient.

- ☐ **Arterial cannulation** - placement of a catheter in an artery in the wrist, arm, or groin to measure blood pressure and obtain blood samples.
- ☐ **Central venous cannulation** - placement of a catheter in a vein in the chest, neck, or groin to administer fluids and medication and to obtain blood samples.
- ☐ **Pulmonary artery/right heart catheterization** - placement of a catheter in a vein in the chest to measure heart pressures and to monitor heart function.
- ☐ **Fiberoptic bronchoscopy** - passage of a flexible probe through the breathing tube and into the airway to inspect the lungs, suction secretions, and collect fluid samples.
- ☐ **Lymph node biopsy** - surgical removal of a lymph node under local anesthesia.
- ☐ **Medication administration** - medicines will be used to both assure comfort and improve the function of potentially transplantable organs.

Medications used to assure comfort include morphine and other analgesics. These medicines reduce discomfort as life support is withdrawn but may have the unintended consequence of suppressing breathing and hastening death.

The following medications, if checked, may be used to improve the function of potentially transplantable organs and will be administered once care is transitioned to comfort-focused care only and agreed upon by the New England Donor Services (NEDS) donation team including:

- ☐ Heparin or alternative anticoagulant (a blood thinner)
- ☐ Phentolamine (a medicine that helps regulate blood flow to vital organs), and
- ☐ Steroids (which decrease inflammation and improve organ function).

Alternatives to Treatment: The doctor has spoken with me about the reasons for the procedures outlined above, which are designed to maintain comfort and dignity at the end of life, while increasing the chances of successful organ donation and transplantation. The doctor has also spoken about the voluntary nature of organ donation and has explained other options for the care of the patient.

Risks: The doctor has explained that there are some risks associated with these procedures. The risks include, but are not limited to, allergic reactions, drug reactions, bleeding, blood clots, infection, and the need for more procedures or treatments. The doctor has also explained that there may be other risks or complications not listed here. When possible, I understand that the physicians will attempt to inform me of any additional risks associated with donor management.

Consent: My questions have been answered. I have read and understood the content of this form. I consent to the treatment of the patient as outlined above.

X _____ X _____ X ____ / ____ / ____
Person authorized to sign for patient and Relationship to patient Date

I have explained the above statements and answered all questions.

X _____ X _____ X ____ / ____ / ____
Signature of Physician / Healthcare Provider Print Name Date

Maastricht Classification – Definition of Controlled DCD Donors

DCD donors are grouped by the Maastricht classification (1995; amended 2003):

- I. Dead on arrival to hospital
- II. Unsuccessful resuscitation
- III. Awaiting cardiac arrest – In-Patient (withdrawal of support)
- IV. Cardiac arrest after brain-stem death
- V. Cardiac arrest in a hospital inpatient

Controlled DCD donors would include those outlined in classification III of the Maastricht criteria.

The Uniform Determination of Death Act

The National Conference of Commissioners on Uniform State Laws in 1980 formulated the Uniform Determination of Death Act (UDDA).

The UDDA states that: “An individual who has sustained either

- (1) Irreversible cessation of circulatory and respiratory functions, or
- (2) Irreversible cessation of all functions of the entire brain, including the brain stem is dead.

A determination of death must be made in accordance with accepted medical standards. This definition was approved by the American Medical Association in 1980 and by the American Bar Association in 1981. Today all fifty states and the District of Columbia follow the UDDA as a legal standard of death.

Sources:

- President’s Commission for the Study of Ethical Problems in Medicine and Biomedical and Behavioral Research, Defining Death: A Report on the Medical, Legal and Ethical Issues in the Determination of Death (Washington: Government Printing Office, 1981), p.73
- Uniform Determination of Death Act. 12 Uniform Laws Annotated 320 (1990 Supp).